

العمر(تاريخ الميلاد) : 13-10-1991 المهنة : بدون عمل

التوقيع: (_____)

التعليق على التشخيص : -----

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

----- / د ----- / د ----- / د. | ----- / د. |

التشخيص النهائي :
 خطة العلاج :
 المستشفى :
 قسم :
 نفقات :
 ملاحظات :

رئيس
المجلس الطبي المتخصص

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Name: فاطمه عادل عبدالتني عبدالحكم National ID: 29110132101026	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
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المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

فاطمة
عادل عبد النبي عبد الحكيم
قرية ميت رهينة
مركز البدرشين - الجيزة

٢٩٩١/١ ٢٩٩١.١٣٢١.١٠.٢٦

JK7367640

٢٠٢٢/٠٦ ٢٩٩١.١٣٢١.١٠.٢٦

ربة منزل

أنثى مسلمة متروجة

شريف عبدالناصر عبدالكريم سيد عيسى

البطاقة سارية حتى ٢٠٢٩/٠٦/١٨

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DMs
فيما را
زوكادكس



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

**MDT form**

A-Patient ID:

1- Name : فاطمة محمد عبد النبي عبد الحكيم

Sex:

3- Medical number:

Female

83179472

Old Med :

Old Med :

2-ID number: 29110132101026

Patient Age : 32 Y



Family history

Family History

-VE

Phone

Phone

BMI

BMI

REASON OF MDT

REASON OF MDT

NEW

marital status

Marital Status

MARRIED

NRS Score

NRS Score

GROUP

GROUP

C

Date of MDT

Date Of MDT

29/01/2024

Co morbidity

Co Morbidity

-VE

Menopausal status

Menopausal Status

Initial STAGE

Initial STAGE

Advanced

Clinical data

Breast Imaging :

Mets Workup :

CT CAP ---> FREE

BONE SCAN REQUESTED

BX
IDCER
8/8PR
8/8Her 2 Neu
2SISH
Not AmplifiedKi67
20%

BX COMMENT:

ER COMMENT:

PR COMMENT:

Her 2 Neu COMMENT:

SISH COMMENT:

T
cT4cN
1M
0

Sonomammography

PATIENT HISTORY

Bone scan

Echo, labs

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

For NACT ACx4 then Taxotere x4 then MRM (in view of multicentricity)

SIGNATURE

MEDICAL ONCOLOGY

MEDICAL ONCOLOGY

RADIOLOGY

RADIOLOGY

SURGERY

SURGERY

PATHOLOGY

PATHOLOGY

Psychiatric

Psychiatric

Nutrition

Nutrition



Patient Name	: الأستاذة / فاطمة عادل عبدالنبي عبدالحكيم	Request No.	: 13009789
Patient No.	: 29110132101026	Request Date	: 09/12/2023
Path No.	: 4378/23	Verification Date	: 28/12/2023
Sex	: Female	Financial Category	: مبادئ - مبادئ
Age	: 32 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Right breast mass

Nature of Specimen: Tru-cut biopsy.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

- * IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 80% of tumor cells (Allred score 8/8).
- PR: Strong nuclear reaction in 80% of tumor cells (Allred score 8/8).
- Her2: Moderate complete membrane reaction in >10% of tumor cells (score 2+).
- SISH: Not amplified.
- (Average Her2 copy No.: 2.6, Her2/CEP17 ratio: 1.4)
- Ki67: 20% nuclear reaction of tumor cells.

Conclusion:

Right breast mass, Tru-cut biopsy:

Invasive duct carcinoma, Grade II with cribriform features.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Equivocal (score 2+).
- SISH: Not amplified.
- Ki67: Positive in 20% of tumor cells.

Verified by :

Dr.Marwa mahmoud





Patient Name : الأستاذة / فاطمة عادل عبدالنبي عبدالحكيم	Request No. : 13009789
Patient No. : 29110132101026	Request Date : 09/12/2023
Path No. : 4378/23	Verification Date : 28/12/2023
Sex : Female	Financial Category : اورام دار السلام - مبادره
Age : 32 Y	Department : Out Patient
Referreing Doc :	Registered by : .marwa mohamed youss

Pathology Report

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Prof. Dr. Iman Gouda

Dr. Menna Nabil

Verified by :

Dr.Marwa mahmoud





Patient Name: Fatma Adel Abd A. Naby Abd Al Hakeem	Date : 14/01/2024
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 32 Year(s)
Medical ID: 4857/2023	46061

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

1. Chest:

- Right axillary level I and II pathologically enlarged lymph nodes the largest measures 1.3 X 0.8 cm.
- Few bilateral scattered tiny pulmonary nodules reaching 3 mm.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is average in size with homogenous CT texture. No definite focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

3. The scanned skeleton:

- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.

Name :Fatma Adel Abd El Naby
Physician: Prof.Dr.Ahmed Farag
Exam date : 3/1 / 2024

اليمين

RIGHT BREAST HIGH RESOLUTION ULTRASOUND
EXAMINATION REVEALED:

- Right UQQ irregular shaped infiltrative hypoechoic solid mass lesion is seen at 9-12 O'clock, about 1 cm from the areola measuring 69x55x22 mm in diameter ,being hypovascular upon Color Doppler application with pathological microcalcifications within. It is seen abutting the pectoralis muscle with no sonographic evidence of deep infiltration. Surrounding architectural distortion is noted.
- A similar lesion is seen at the UIQ , at 1 O'clock ,bout 4 cm from the areola ,measuring 26x20 mm in diameter.
- Two other smaller lesions are seen at :
 - 2 O'clock ,about 1 cm from the areola ,measuring 14x8 mm in diameter.
 - 7 O'clock ,about 1 cm from the areola ,measuring 5x4 mm in diameter .
- Irregular nodular areola skin thickening is noted with mildly dilated retroareolar ducts.
- Multiple pathologically enlarged right axillary level I and II LNs are noted ,reaching 17x7 mm and 13x9 mm in diameter with thickened cortex and eccentric hilum.
- No pathological infra or supraclavicular LNs.

CONCLUSION:

Right breast multicentric malignant looking solid mass lesions with pathological microcalcifications and right axillary pathological lymphadenopathies **BIRADS 5** ,for U/S guided Tru Cut biopsy.

Thank you.
DR., Sara Mahmoud,MD